

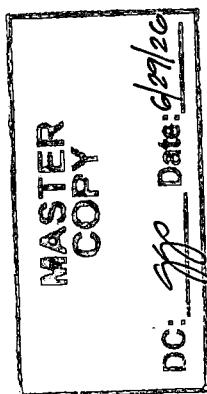
PHILHEALTH CIRCULARNo. 2026 - 0008

**TO : ALL ACCREDITED HEALTH FACILITIES AND
HEALTHCARE PROFESSIONALS, PHILHEALTH
REGIONAL OFFICES, AND ALL OTHERS CONCERNED**

SUBJECT : Benefits Package for Moderate to Severe Leptospirosis

I. RATIONALE

PhilHealth provides coverage for leptospirosis needing inpatient admission through the All Case Rates (ACR). However, despite increasing reimbursement rate of around 80% across the board for the ACR in CY 2024 the cost of hospitalization for moderate to severe leptospirosis entails significant out-of-pocket costs of as high as 90% based on CY 2025 claims data, where 80% of leptospirosis claims come from government facilities. Moreover, around 4% of leptospirosis claims in CY 2025 needed some form of additional procedures, such as renal replacement therapy (RRT), invasive mechanical ventilation, and intensive care monitoring.



In line with PhilHealth's mandate to provide responsive benefits that address the needs of its members pursuant to Republic Act (R.A.) No. 7875, as amended by R.A. Nos. 9241 and 10606 (National Health Insurance Act of 2013), as well as the financing reforms under R.A. No. 11223 or the Universal Health Care Act, and in preparation for the transition to a new provider payment mechanism, from the All Case Rates (ACR) to diagnosis-related groups (DRG), PhilHealth has identified moderate to severe leptospirosis as a priority condition under the ACR. This prioritization aims to strengthen financial coverage and protect members from catastrophic health expenses associated with the illness. Accordingly, the PhilHealth Board of Directors, through Board Resolution No. 3088, s. 2026, approved the expanded coverage for moderate to severe leptospirosis in the inpatient setting.

II. OBJECTIVES

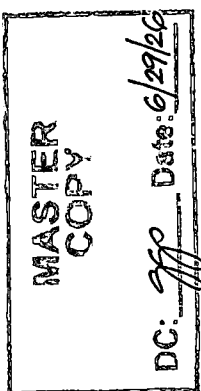
- A. Rationalize the case rates for the inpatient management of moderate to severe leptospirosis to be more responsive to current hospital costs;
- B. Reduce patient out-of-pocket spending; and
- C. Support cost containment for quality care provided in accredited hospitals.

III. SCOPE

This PhilHealth Circular covers essential services or the minimum standards of care related to the expansion of moderate to severe leptospirosis coverage, which shall apply to all accredited levels 1 to 3 Health Facilities, PhilHealth Regional Offices (PROs), and all other stakeholders involved in implementing the benefit package for moderate to severe leptospirosis in the hospital setting.

IV. DEFINITION OF TERMS

- A. Balance Billing¹** - additional payments by insured patients on top of the amount paid by insurance when the provider's charges exceed the amount covered by health insurance. Due to financial and service coverage decisions, balance billing may result in increased financial burdens and limited access to health services by households.
- B. Basic or Ward Accommodation²** - the provision of regular meals, a bed in a shared room with 3 or more occupants, with at least fan ventilation, and shared toilet and bath.
- C. Blended Provider Payment Mechanism³** - a healthcare reimbursement approach combining multiple payment models that aims to balance incentivizing preventative care, efficiency, and quality while ensuring financial sustainability and equitable resource allocation.
- D. Copayment⁴** - a predetermined amount that is chargeable to patients to cover the share for amenities, specialist fees for patients admitted to non-basic or non-ward accommodation, choice of physician, or any additional or upgraded services during the episode of inpatient care before service access to manage moral hazards and adverse incentives. Copayment is an example of an out-of-pocket payment or cost-sharing mechanism intended to share the cost of healthcare between the insured and the insurer.
- E. Cross-subsidization⁵** - the economic practice in the Philippine healthcare system of promoting social solidarity by charging higher premium rates to formal employees and higher economic quintiles to subsidize indigents, senior citizens, persons with disabilities, and other disadvantaged populations.



¹PC 2024-0032: Benefits Package for Ischemic Heart Disease - Acute Myocardial Infarction

²DOH-AO 2021-0015, "Standards on Basic and Non-Basic Accommodation in All Hospitals."

³<https://economicsbydesign.com/health-system/health-financing/health-financing-provider-payment-models/#blended-payment-models>

⁴PC 2024-0032: Benefits Package for Ischemic Heart Disease - Acute Myocardial Infarction

⁵Learner's Material on the Philippine Health Insurance Corporation, 2015. PhilHealth Social Health Insurance Academy and Department of Education.

- F. Efficiency Gains⁶** - may be referred to reducing administrative waste, lowering service prices, or optimizing resource use to achieve better health outcomes without increasing spending and improving overall health system sustainability.
- G. Leptospirosis⁷** - a zoonotic infection caused by pathogenic spirochetes of the genus *Leptospira*. Leptospirosis is primarily a disease of domesticated and wild animals. Humans are infected through direct and indirect contact with these animals. The source of infection is water or soil contaminated with infected urine, or tissues from infected animals.
- H. Mild Leptospirosis⁸** - a category of leptospirosis presenting with stable vital signs, anicteric sclerae, no jaundice, with good urine output, no evidence of meningismus / meningeal irritation, no dyspnea, no tachypnea, no hemoptysis, no bleeding, not in sepsis / septic shock, and can be resolved with oral medications.
- I. Moderate to Severe Leptospirosis⁹** - a category of leptospirosis characterized by unstable vital signs, jaundice or icteric sclerae, abdominal pain, nausea, vomiting and diarrhea, oliguria or anuria, bleeding, meningismus or meningeal irritation, sepsis / septic shock, altered mental status, difficulty of breathing or hemoptysis that are managed in a hospital setting for proper diagnosis and appropriate monitoring and medical management.
- J. Out-of Pocket (OOP)¹⁰** - balance of healthcare provider charges paid directly by the patients from their own resources or cash reserves for health made directly from households to providers.
- K. Renal Replacement Therapy (RRT)¹¹** - a medical treatment that replaces the normal kidney function in patients with acute or chronic kidney failure. It involves using various techniques, such as hemodialysis, peritoneal dialysis, and kidney transplantation, to remove waste products, excess fluids, and electrolytes from the bloodstream.

V. POLICY STATEMENTS

- A.** PhilHealth identified leptospirosis as one of the priority conditions based on disease burden to improve financial coverage and ensure financial risk protection while transitioning its provider payment mechanism from ACR to DRG.

⁶Eisenhauer, J. G. (2009). Efficiency Gains from Health Insurance. pp. 117-126

⁷DOH Guidelines for Leptospirosis for Hospitals, 2019 edition

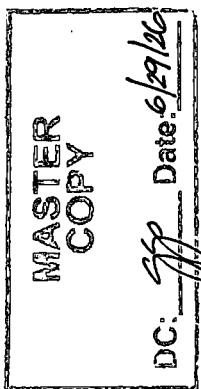
⁸Ibid

⁹Ibid

¹⁰Ibid.

¹¹PhilHealth Circular 2024-0014 "Institutionalization of 156 Hemodialysis Sessions and Coverage Expansion (Revision 1)

- B. PhilHealth utilizes a blended provider payment mechanism to reimburse HFs for delivering the minimum standards of care for moderate to severe leptospirosis and for treatments for its medical complexity.
- C. The minimum standards of care recommendations from the Department of Health's (DOH) clinical practice guidelines (CPG)¹², in collaboration with medical societies and DOH hospitals, serve as the basis for the service coverage and costing analysis of the benefits package for moderate to severe leptospirosis.
- D. PhilHealth shall reimburse drugs listed in the latest edition of the Philippine National Drug Formulary (PNDF). PhilHealth reiterates the mandate of the Health Technology Assessment Council (HTAC), which provides positive recommendations for any proposal to cover drugs, medicines, or biologicals not listed in the latest PNDP, new health technologies, procedures, and other treatment interventions.
- E. PhilHealth supports continuous quality improvement efforts to enhance leptospirosis prevention and care in the country, strengthen referral networks aligned with Universal Health Care, and promote collaboration to standardize local practices and improve access to quality services in accredited HFs.
- F. With the rate adjustment for moderate to severe leptospirosis and the increase in PhilHealth coverage, all PhilHealth beneficiaries shall be entitled to no copayment when admitted to basic or ward accommodation following the DOH-Administrative Order¹³ in public and private HFs.
- G. Accredited public and private hospitals shall not balance bill or charge patients admitted to basic or ward accommodation for essential services covered by PhilHealth under this policy.
- H. Treatment and management costs exceeding the reimbursement rates for moderate to severe leptospirosis may be offset through cross-subsidization using alternative funding sources, efficiency gains, or out-of-pocket (OOP) spending.
- I. Services provided that are beyond the PhilHealth coverage, such as amenities, choice of physician, upgrade of services, or other services that are not covered in this policy, shall be charged a maximum copayment of up to 20% of the package rate for admissions in non-basic accommodation.
- J. The reimbursement rates for moderate to severe leptospirosis represent the average cost of managing an episode of care depending on its medical complexity and corresponding treatment. These amounts may differ from the actual hospitalization charges reflected in the Statement of Account (SOA), as the actual cost of care per patient may be higher or lower than the published reimbursement rate. As such, PhilHealth maintains the flexibility

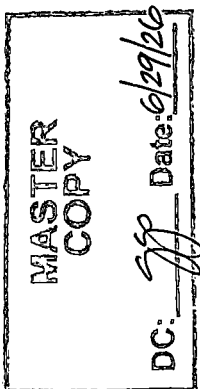


¹²DOH Guidelines for Leptospirosis for Hospitals, 2019 edition

¹³DOH-AO 2021-0015, "Standards on Basic and Non-Basic Accommodation in All Hospitals

to adjust payment rates based on the specific services provided and the efficiency with which healthcare facilities deliver quality healthcare services.

- K. As part of informed consent, the attending physician should properly inform patients and/or their relatives of the minimum/standard services for moderate to severe leptospirosis covered under this policy.
- L. All accredited HF's should maintain minimum stock levels of essential medications, life-saving drugs, IV fluids, and supplies at all times to ensure the timely delivery of quality healthcare services and discourage unwarranted OOP expenses from outside purchases of medications and other essential services for leptospirosis.
- M. As stipulated in the UHC Act, Chapter IV, Section 18 (b), there will be no differentiation between facility and professional fees. PhilHealth shall credit all payments to the accounts of accredited private and public HF's. In the case of government HF's, it is the sole responsibility of the HF to distribute professional fees (PF) to attending physicians or health workers in accordance with its internal agreements and processes.
- N. Accredited HF's shall follow the DOH-endorsed CPG for leptospirosis¹⁴ to manage patients and ensure adherence by medical professionals who are appropriately credentialed and privileged to practice in the HF's, as well as all hospital staff in charge of patients with moderate to severe leptospirosis.
- O. Accredited HF's that lack the service capability to manage moderate to severe leptospirosis, except HF's in geographically isolated and disadvantaged areas (GIDA), shall properly coordinate and facilitate the timely referral of patients after providing standard emergency and life-saving measures to capable higher-level HF's.
- P. PhilHealth Benefits Package for Leptospirosis



1. Chapter II, Section 5 of R.A. No. 11223 stipulates that, "Every Filipino citizen shall be automatically included in the National Health Insurance Program (NHIP)." Thus, all Filipinos are eligible to avail themselves of the benefits package for moderate to severe leptospirosis. Therefore, PhilHealth reiterates that claims submission does not require a printed copy of the member data record (MDR). All accredited HF's should deduct PhilHealth benefits on any day of the week at any time upon patient discharge.
2. Accredited HF's shall ensure the delivery of the minimum standards of care for managing moderate to severe leptospirosis according to DOH-endorsed CPG¹⁵ recommendations applicable in local practice. In addition, they shall ensure the quality and availability of drugs and medicines, functioning laboratory equipment, and other needed

¹⁴DOH Guidelines for Leptospirosis for Hospitals, 2019 edition

¹⁵Ibid.

equipment, as well as timely laboratory chemistry services and adequate human resources.

3. Table 1 lists the annexes for PhilHealth's coverage for moderate to severe leptospirosis and moderate to severe leptospirosis requiring add-on procedures.

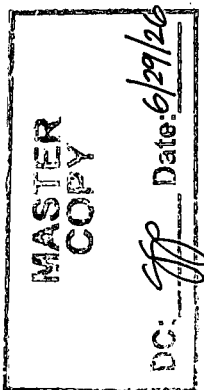
Annex	Description
A.1	Covered Services for Moderate to Severe Leptospirosis
A.2	Covered Services for Moderate to Severe Leptospirosis Requiring Hemodialysis and Vascular Access
A.3	Covered Services for Moderate to Severe Leptospirosis Requiring Peritoneal Dialysis (PD) and PD Solutions
A.4	Covered Services for Moderate to Severe Leptospirosis Requiring Hemodialysis and Hemoperfusion
A.5	Covered Services for Moderate to Severe Leptospirosis Requiring Hemodialysis, Invasive or Non-Invasive Ventilatory Support (e.g., Mechanical Ventilation), and Pulmonary Rehabilitation

Table 1: List of Covered Services for Moderate to Severe Leptospirosis And Moderate to Severe Leptospirosis Requiring Add-on Procedures

4. Table 2 shows the package code, description, and corresponding package rate for moderate to severe leptospirosis and moderate to severe leptospirosis requiring add-on procedures.

Package Code	Description	Package Rate (Php)
LEPT1	Leptospirosis, moderate to severe	63,000
LEPT2	Leptospirosis, moderate to severe, requiring hemodialysis (HD) and vascular access	170,000
LEPT3	Leptospirosis, moderate to severe, requiring peritoneal dialysis (PD) and PD Solutions	230,000
LEPT4	Leptospirosis, moderate to severe, requiring HD and hemoperfusion (HP)	325,000
LEPT5	Leptospirosis, moderate to severe, requiring HD, invasive or non-invasive ventilatory Support (e.g., mechanical ventilation), and pulmonary rehabilitation	411,000

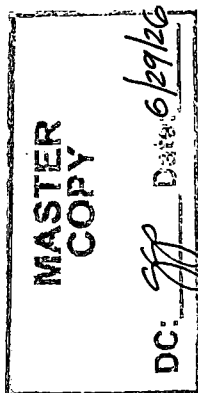
Table 2: Package Code, Description, and Package Rate for Moderate to Severe Leptospirosis and Moderate to Severe Leptospirosis With Add-on Procedures



5. Coverage for services for mild leptospirosis managed in the primary or emergency care setting not requiring inpatient admission shall be availed of under YAKAP¹⁶ or its succeeding amendments, PhilHealth GAMOT¹⁷ or its succeeding amendments, and the OECB¹⁸ Package or its succeeding amendments.
6. PhilHealth marketing and communications shall highlight the preventive approach and post-exposure antibiotic prophylaxis for leptospirosis, as services for the early detection and treatment of leptospirosis are covered under PhilHealth YAKAP and GAMOT.
7. Diagnosis of Leptospirosis shall follow the criteria stipulated in the DOH Guidelines for Leptospirosis in Hospitals¹⁹ or its subsequent revisions and amendments. Initiation of treatment shall not be delayed pending confirmatory testing, including results of the microagglutination test. Clinical diagnosis, particularly in the presence of relevant epidemiologic risk factors and a high index of suspicion, shall take precedence.²⁰

Q. Claims Filing

1. Accredited HFs shall strictly follow current PhilHealth policies on claims submission, including correct ICD-10 coding of the primary diagnosis (Table 3: ICD-10 Code and Description for Leptospirosis) with the corresponding procedure coding, if there is an add-on procedure (Table 4: RVS Codes and Descriptions related to Leptospirosis); submission of properly accomplished PhilHealth claim forms (CF); electronic Statement of Account (eSOA); and other data and documentary requirements as stipulated in existing policies.



ICD-10 Code	Description
A27.0	Leptospirosis Icterohaemorrhagica; Leptospirosis due to <i>Leptospira Interrogans</i> Serovar Icterohaemorrhagiae
A27.8	Other forms of Leptospirosis
A27.9	Leptospirosis, Unspecified

Table 3: ICD-10 Codes and Descriptions for Leptospirosis

RVS Code	Description
90935	Hemodialysis Procedure
90945	Dialysis Procedure Other Than Hemodialysis (e.g., Peritoneal, Hemofiltration)

¹⁶PC No. 2025-0017: Selection and Empanelment for Philhealth's Primary Care Benefit Package

¹⁷PC No. 2026-0002: Supplemental Guidelines for the Nationwide Implementation of PhilHealth Guaranteed and Accessible Medications for Outpatient Treatment

¹⁸PC No. 2025-0020: Implementing Guidelines for the PhilHealth Outpatient Emergency Care Benefits

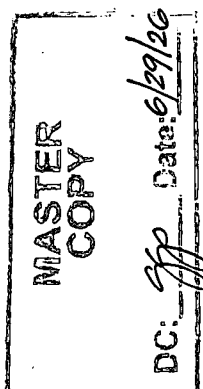
¹⁹DOH Guidelines for Leptospirosis for Hospitals 2019

²⁰Ibid.

RVS Code	Description
36488	Placement of Central Venous Catheter (Subclavian, Jugular, or other vein) (e.g., for Central Venous Pressure, Hyperalimentation, Hemodialysis, or Chemotherapy); Percutaneous or Cutdown

Table 4: RVS Codes and Descriptions Related to Leptospirosis

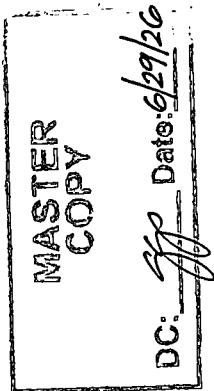
2. The appropriate ICD-10 and/or RVS code/s shall be indicated in Claim Form 2 (CF2) Part II, Item no. 7.
3. The appropriate package code shall be indicated in CF2, Part II Item 9a, and in Part V Item 1.1 in the Claim Signature Form (CSF).
4. Accredited HFs shall indicate the patient OOP spending, such as purchases or expenses, and/or copayment of the member/patient and other funding sources in CF2 Part III on Consumption of Benefits and in the eSOA following PC No. 2023-0026 [Electronic Data Submission of the Statement of Account (SOA) for All Case Rates (ACR) Claims and Identified PhilHealth Benefits (Revision 1)] or its subsequent revisions.
5. Accredited HFs shall follow all relevant laws, such as R.A. No. 9994 or the Expanded Senior Citizens Act of 2010 and R.A. No. 10754 or An Act Expanding the Benefits and Privileges of Persons with Disability, including prospective laws providing mandatory discounts, guidelines of the Bureau of Internal Revenue (BIR), and the order of charging from funding sources based on the DOH-DSWD-PCSO-PHIC Joint Administrative Order No. 2020-0001 (Operational Guidelines for the Implementation of the Medical and Financial Assistance to Indigent and Financially-Incapacitated Patients Pursuant to Republic Act No. 11463 also known as "Malasakit Centers Act of 2019").
6. With this, PhilHealth benefits and all mandatory discounts provided by law, such as, but not limited to, senior citizen and PWD discounts, shall be deducted first from the patient's total hospital bill. Benefits from private health insurance (PHI), health maintenance organizations (HMO), or employee benefits shall be applied after PhilHealth deductions and complement the PhilHealth benefits packages.
7. Accredited HFs shall file all claims to PhilHealth within the prescribed filing period of sixty (60) calendar days. Direct filing by members/beneficiaries is not allowed, except in cases of claims arising from confinements abroad, following the existing guidelines of PhilHealth on direct filing.
8. PhilHealth shall process and pay claims for confinement abroad based on the remaining balance not covered by any additional insurance or incurred as OOP expenses, provided that such balance does not exceed the published reimbursement rates set forth in this policy.



9. Rules on late filing shall apply, except when the delay in filing claims is due to natural calamities or other fortuitous events, following the existing guidelines of PhilHealth on the provision of special privileges to those affected by fortuitous events shall apply.
10. Accredited HF's may file a motion for reconsideration (MR) and appeal for claims denied by PhilHealth following existing policies.
11. Accredited Levels 1 to 3 HF's shall provide a detailed account of the patient's clinical history, physical examination (PE) findings, course in the ward, and a summary of the pertinent laboratory and diagnostic results on the PhilHealth Claim Form 4 (CF4).
12. Accredited Levels 1 to 3 HF's shall attach a Certificate of Renal Replacement Therapy Add-On Services for Patients with Moderate to Severe Leptospirosis (Annex B: Certificate of Renal Replacement Therapy Add-on Service For Patients With Moderate To Severe Leptospirosis) for claims applications for moderate to severe leptospirosis needing hemodialysis, peritoneal dialysis, or hemodialysis combined with hemoperfusion.
13. Accredited Levels 1 to 3 HF's shall attach a Certificate of Invasive or Non-Invasive Ventilatory Support Add-On Service for Patients with Moderate to Severe Leptospirosis (Annex C: Certificate of Invasive or Non-invasive Ventilatory Support As Add-on Service For Patients With Moderate To Severe Leptospirosis) and the Certificate of Pulmonary Rehabilitation Add-On Service for Patients with Moderate to Severe Leptospirosis (Annex D: Certificate Of Pulmonary Rehabilitation Add-On Service For Patients With Moderate To Severe Leptospirosis) for pulmonary rehabilitation services rendered.

R. Claims Evaluation and Payment

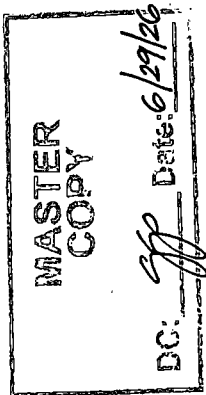
1. PhilHealth shall reimburse accredited HF's with service capability for the management of moderate to severe leptospirosis based on the amount reflected on the eSOA up to the package rate corresponding to the package code indicated on CF2 Part II item 9a and CSF Part V item 1.1. There shall be no second case rate.
2. PhilHealth shall reimburse claims for moderate to severe leptospirosis requiring hemodialysis, peritoneal dialysis, or hemodialysis combined with hemoperfusion, with the photocopy of the corresponding treatment sheet attached to the CF4.
3. PhilHealth shall reimburse claims for moderate to severe leptospirosis requiring invasive or non-invasive ventilatory support (e.g., mechanical ventilation) and pulmonary rehabilitation, with attached Certificate of Invasive or Non-Invasive Ventilatory Support As Add-on Service for Patients with Moderate to Severe Leptospirosis (Annex C) and Certificate of Pulmonary Rehabilitation Add-On Service for Patients with Moderate to Severe Leptospirosis (Annex D) attached to the CF4.



4. PhilHealth reserves the right to subject any or all claims to medical review before and/or after payment or reimbursement of accredited HF's following existing rules.
5. PhilHealth shall apply the "return to hospital (RTH)" policy to claims with incomplete and/or discrepant requirements for compliance within the prescribed period.
6. PhilHealth shall pay accredited HF's that refer moderate to severe leptospirosis patients to higher-level HF's using RVS code "P0001-Referral Package" following Annex B of PC No. 2024-0037 [Fifty Percent (50%) Adjustment of Selected Case Rates] or its subsequent amendments.
7. Accredited HF shall not balance bill or charge an additional payment for the covered services for patients with a patient disposition of "expired." These claims shall be paid based on the amount reflected on the eSOA up to the corresponding package rate for moderate to severe leptospirosis or moderate to severe leptospirosis requiring add-on procedures.
8. Claims for leptospirosis of accredited levels 1 to 3 HF's with a patient disposition of "Home Against Medical Advice (HAMA)" or "absconded" indicated in CF2 shall be paid by PhilHealth the amount stated on the eSOA, but not to exceed the package rates set forth in this policy. PhilHealth shall closely monitor accredited HF's submitting claims with a patient disposition of HAMA or absconded.

S. Monitoring

1. PhilHealth shall enforce its current policies and rules for monitoring the performance of accredited HF's in implementing the benefits package for moderate to severe leptospirosis and establish strict control mechanisms to ensure quality healthcare delivery and financial risk protection, as well as prevent adverse provider behaviors and non-compliance with existing rules.
2. PhilHealth shall conduct regular utilization reviews to determine outliers and sources of out-of-pocket expenses for moderate to severe leptospirosis and moderate to severe leptospirosis requiring add-on procedures.
3. Appropriate offices shall develop the necessary tools to monitor compliance to standards of care and implementation rules.
4. PhilHealth encourages collaboration with content experts in reviewing and conducting appropriate studies of the actual practices of healthcare providers during the implementation of this policy.



5. PhilHealth beneficiaries are strongly encouraged to provide feedback or report concerns about the implementation of this policy or their experiences with benefit availment to the Corporate Action Center (CAC) via the hotline (02) 8862-2588 or email actioncenter@philhealth.gov.ph.

T. Marketing and Promotion

PhilHealth shall conduct communication and social marketing activities, in collaboration with experts and stakeholders, to educate healthcare providers and the public on increasing their awareness of the coverage for leptospirosis following the current Social Marketing and Communication Plan (SMCP).

U. Policy Review

PhilHealth shall conduct a policy review of the benefits package for moderate to severe leptospirosis in parallel with the development and transition to the DRG provider payment mechanism, in collaboration and consultation with key stakeholders, experts, and implementers.

This PhilHealth Circular shall be enhanced as necessary based on the results of the policy review.

VI. PENALTY CLAUSE

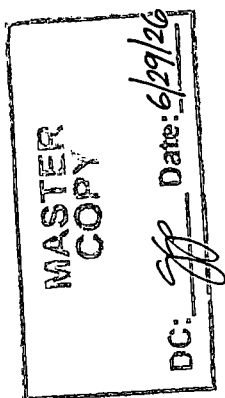
Any violation of this PhilHealth Circular shall be dealt with and penalized in accordance with pertinent provisions of R.A. No. 7875, as amended by R.A. Nos. 9241 and 10606 (National Health Insurance Act of 2013), R.A. No. 11223 (Universal Health Care Act), and their respective Implementing Rules and Regulations, including the PhilHealth Rules on Administrative Cases (PROAC).

VII. TRANSITORY CLAUSE

Claims filed with admission dates prior to the date of effectivity of this PhilHealth Circular shall be paid the amount of Php 21,450, following the amount stated in Annex A of PC No. 2024-0037 entitled, "Fifty Percent (50%) Adjustment of Select Case Rates."

VIII. SEPARABILITY CLAUSE

In the event that any part or provision of this PhilHealth Circular is declared unauthorized or invalid by any court of law or competent authority, those provisions not affected by such declaration shall remain valid and effective.

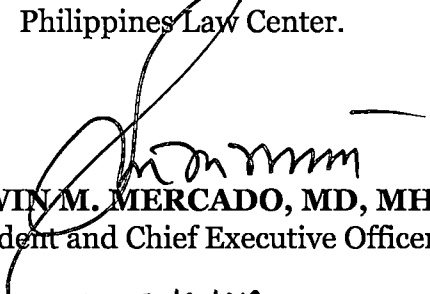


IX. REPEALING CLAUSE

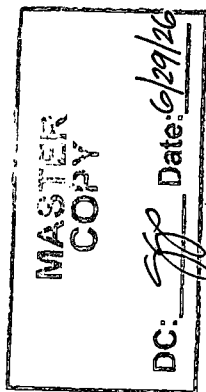
All PhilHealth Circulars, issuances, rules, and regulations or parts thereof that are contrary to and inconsistent with this PhilHealth Circular are hereby repealed, amended, or modified accordingly.

X. DATE OF EFFECTIVITY

This PhilHealth Circular shall take effect fifteen (15) days after its publication in the Official Gazette or in any newspaper of general circulation unless it is otherwise provided. Three (3) certified true copies shall be filed with the Office of the National Administrative Register (ONAR) of the University of the Philippines Law Center.


EDWIN M. MERCADO, MD, MHA, MSSc
President and Chief Executive Officer

Date signed: 06/25/2026

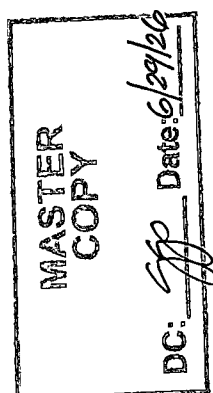


Annex A.1:

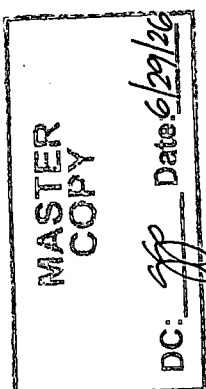
Covered Services for Moderate to Severe Leptospirosis

The items listed as covered services are given based on the healthcare provider's clinical

Package Inclusions	Minimum Coverage
Room and Board	Basic Accommodation
Drugs/Medicines	Amino Acid Solution for Parenteral Nutrition Calcium Gluconate Solution Ceftriaxone Cyclophosphamide Dextrose 5% in Water Dextrose 50% in Water Doxycycline Hydrocortisone Isotonic Electrolyte Solution for IV Infusion Methylprednisolone Omeprazole Paracetamol Plain Normal Saline Solution Potassium Chloride Solution Sodium Bicarbonate Solution
Laboratory Tests and Diagnostics	Alanine aminotransferase (ALT) Albumin (Alb) Arterial Blood Gas (ABG) Aspartate aminotransferase (AST) Blood Typing Blood Urea Nitrogen (BUN) C-Reactive Protein (CRP) Capillary Blood Glucose (CBG) Chest X-ray Complete Blood Count (CBC) Creatinine (Crea) Dengue IgG* Dengue IgM* Dengue NS1* Direct Bilirubin (Dbil) Erythrocyte Sedimentation Rate (ESR) Free Thyroxine (FT4) Free triiodothyronine (FT3)



Package Inclusions	Minimum Coverage
	Hemoglobin A1C (HBA1c) Hepatitis B surface antigen (HBsAG) Hepatitis C antibody test (Anti HCV) Indirect Bilirubin (Ibil) Ionized Calcium (iCa) Latex Agglutination Test IgG (LAT IgG) Latex Agglutination Test IgM (LAT IgM) Magnesium (Mg) Microscopic Agglutination Test (Lepto MAT) Partial thromboplastin time (PTT) Phosphorus (P) Potassium (K+) Prothrombin time (PT) Serum Amylase Serum Lipase Sodium (Na) Thyroid-Stimulating Hormone (TSH) Total Bilirubin (Tbil) Urinalysis Whole Abdomen Ultrasound
Supplies and Use of Equipment	Alcohol swab Blood pressure monitor Cardiac monitor Chlorhexedine Cotton balls Foley Catheter Gauze IV Catheters IV Tubings Micro/Macro set Oxygen cannula/ oxygen mask Pulse oximeter Sterile gloves Transparent Film Dressing
Pulmonary Services	Oxygen
Cardiology	ECG

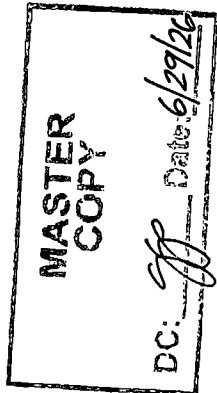


Annex A.2 :

**Covered Services for Moderate to Severe Leptospirosis
Requiring Hemodialysis and Vascular Access**

The items listed as covered services are given based on the healthcare provider's clinical assessment and specific clinical indications of the patient. The listed services may or may not be provided simultaneously in every case.

Package Inclusions	Particulars
Procedures	IJ Catheter triple lumen insertion and removal
Supplies and Use of Equipment	Dialyzer Hemodialysis prio A Hemodialysis prio B
Laboratory Tests and Diagnostics	Estimated glomerular filtration rate (eGFR)

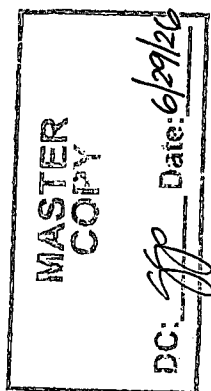


Annex A.3

**Covered Services for Moderate to Severe Leptospirosis
Requiring Peritoneal Dialysis (PD) and PD Solutions**

The items listed as covered services are given based on the healthcare provider's clinical assessment and specific clinical indications of the patient. The listed services may or may not be provided simultaneously in every case.

Package Inclusions	Particulars
Drugs/Medicines	Amoxicillin
Procedure	PD catheter insertion and removal PD exchanges (Cycler-Assisted)
Laboratory Tests and Diagnostics	Estimated glomerular filtration rate (eGFR)

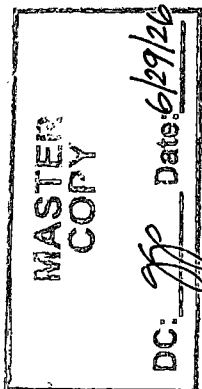


Annex A.4

**Covered Services for Moderate to Severe Leptospirosis
Requiring Hemodialysis and Hemoperfusion**

The items listed as covered services are given based on the healthcare provider's clinical assessment and specific clinical indications of the patient. The listed services may or may not be provided simultaneously in every case.

Package Inclusions	Particulars
Procedure	IJ Catheter triple lumen insertion and removal
Supplies and Use of Equipment	Dialyzer HA 330 Cartridge Hemodialysis prio A Hemodialysis prio B
Laboratory Tests and Diagnostics	2 D ECHO with/ without Doppler Estimated glomerular filtration rate (eGFR)



Annex A.5:

**Covered Services for Moderate to Severe Leptospirosis Requiring Hemodialysis,
Invasive or Non-invasive Ventilatory Support and Pulmonary Rehabilitation**

The items listed as covered services are given based on the healthcare provider's clinical

Package Inclusions	Minimum Coverage
Room and Board	Intensive Care Unit Services
Drugs/Medicines	Piperacillin Tazobactam
Procedures	IJ Catheter triple lumen insertion and removal Intubation
Supplies and Use of Equipment	Dialyzer Hemodialysis prio A Hemodialysis prio B
Laboratory Tests and Diagnostics	Arterial Blood Gas (ABG) Blood Components (Platelets) Blood CS x 2 sites Estimated glomerular filtration rate (eGFR) ETA GSCS Leukodepleted Packed RBC Sputum GSCS
Cardiology services	2 D Echocardiography with / without Doppler
Pulmonary services	BIPAP /Mechanical Ventilation Pulmonary Rehabilitation

